



Policy

Medical Emergency Team (MET) – Code Blue

Reference #: HP135

Scope

Site	Service/Department/Unit	Disciplines
SCGH	Hospital-Wide	All Staff

Introduction

This policy outlines the medical emergency process at Sir Charles Gairdner Hospital (SCGH) for Medical Emergency Team (MET), in responding to an acute deterioration in clinical status.

The purpose of the MET as a Rapid Response System in a hospital is to minimise mortality and morbidity from acute medical emergencies.

Risk Statement

Non-compliance with this policy will:

Breach legislative requirements	☐	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☐	Misconduct	☐
Breach SCGH Mission & Values	☐	Staff Safety	☐
Other:	☐		

Policy Principles

MET attends all central Sir Charles Gairdner Hospital (SCGH) buildings and their entrances, adjoining paths and courtyards, as depicted on the map in [Appendix 1](#). This includes Blocks A, B, C, D, DD, E, F, G, and the adjacent rose garden in between A and DD blocks and the Social Club Gym.

All other areas external to this central catchment area are unable to receive a MET response. First responders will assist where able, arranging transport to the Emergency Department or calling an ambulance if necessary, on 000.

Scope of practice

- The primary responsibility of SCGH is for the inpatient cohort, who may suddenly deteriorate.
- Visitors who are injured or unwell are to be directed to ED.
- Staff and contractors are managed in accordance with NMHS First Aid Policy.

The MET never takes over clinical governance of the patient, as this always lies with the admitting Consultant or their on-call representative.

The MET includes:

- Critical Care Registrar
- Anaesthetic Registrar
- Medical Registrar
- MET Nurse
- Hospital Service Assistant. (The MET transport trolley will be collected for all non-in-patient ward areas).

The Team Leader will be the clinician with the most appropriate clinical skills to do so, and their position made clear to the remainder of the team.

Criteria

A Code Blue may be activated whenever a patient, visitor or staff member located within the catchment area, meets the following calling criteria:

Acute changes in any 1 or more:	Physiology
Airway	• Threatened
Breathing	ALL RESPIRATORY ARRESTS • Respiration rate < 5 • Respiration rate > 36
Circulation	ALL CARDIAC ARRESTS • Pulse rate < 40 • Pulse rate > 140 • Systolic blood pressure < 90 mmHg
Disability (Neurology)	• Sudden fall in level of consciousness (Fall in GCS of > 2 points or scoring P or U on AVPU scale) • Repeated or prolonged seizures
Urine Output	Unexplained fall to < 100mL total over 3 hours

The MET response also forms part of the escalation criteria on the Observation and Response Chart (ORC). This is a single trigger, 4-tiered response designed to recognise and respond to clinical deterioration in a timely and appropriate manner. The response criteria and actions required, up to and including a MET call are mandatory unless appropriate acceptable altered parameters / modifications state otherwise.

See the SCGH Recognising and Responding to Acute Deterioration (RRAD) Policy for further information.

The following components are important considerations to the MET response:

1. All patients must have a clinical management plan, planning for clinical deterioration or end of life care as appropriate
2. Any modifications to MET calling criteria will only be made by Registrar or Consultant, and not made in lieu of an adequate patient assessment and clinical management plan.
3. In the event of a MET call, the Consultant with the primary responsibility for the patient at that time, or their cover, must always be contacted by the Medical MET team leader for input into the management of their patient.

Activation of a Code Blue Medical Emergency Call

To call the Code Blue Medical Emergency Team: Press the Dial 55 and state “Code Blue”
(State your name, location and location of the medical emergency)

- Should the MET not arrive in a timely manner to the medical emergency, reactivate by calling 55 again stating that it is a recall.
- The team is ONLY activated via pager through the switchboard.
- All staff requiring knowledge of MET (Code Blue) activation must carry a pager enabled to receive them (not solely a DECT phone).
- A call for assistance directed erroneously through to the switchboard will be transferred to 000 to enable the first responder to directly give the Ambulance Service the most accurate information.

Compliance, monitoring and evaluation

MET Activity is reported via an Annual Report prepared by the MET Coordinator. This is supplemented by monthly bulletins and reporting to the Emergency Management and RRAD committees.

Related Documents

- [SCGH Recognising and Responding to Acute Deterioration \(RRAD\) Policy](#)
- [NMHS First Aid Policy](#)
- WA Health [Recognising and Responding to Acute Deterioration](#) Policy
- [Recognising and Responding to Acute Deterioration Standard](#)

Authorisation & Version Control

Policy Sponsor	SCGOPHCG Area Director Clinical Services				
Policy Contact	MET Coordinator				
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Standards Applicable:					

For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

This document can be made available in alternative formats on request for a person with a disability.

Appendix 1: MET Response Area Map

